

CLINICAL NOTE – FICTITIOUS PATIENT DATA (FOR TESTING ONLY)

Patient: Tariq Olamide Adeyemi
Date of Birth: 08 February 2001
MRN: 9920384
SSN: 741-55-2263
Address: 14 Elmwood Avenue, Chicago, IL 60611
Phone: (312) 774-5509
Email: tariq.adeyemi01@icloud.com
Referring Physician: Dr. Priya Subramaniam, NPI 8841760293
Facility: Northwestern Memorial Hospital, Chicago

Visit Date: 27 June 2026
Reason for Visit: Fever, rigors, productive cough – community-acquired pneumonia vs. opportunistic infection. Background of newly diagnosed HIV.

HISTORY OF PRESENT ILLNESS

Tariq Olamide Adeyemi is a 25-year-old male presenting with a 5-day history of high-grade fever (Tmax 39.8 degrees C at home), rigors, productive cough with yellow-green sputum, pleuritic right-sided chest pain, and progressive exertional dyspnoea. He was diagnosed with HIV infection 6 weeks ago (CD4 count 112 cells/uL at diagnosis – not yet on ART at time of presentation). His sister Zainab Adeyemi (contact: 312-990-4473) brought him to the ED. His uncle Emmanuel Adeyemi (home: 773-554-8821) was also notified. HIV Clinic follow-up was booked with Dr. Priya Subramaniam (clinic: 312-695-4440) for 04 July 2026 – patient did not attend prior to this admission.

PAST MEDICAL HISTORY

- HIV infection (newly diagnosed 14 May 2026 – CD4 112 cells/uL, VL 88,400 copies/mL)
- No prior AIDS-defining illnesses documented
- No prior opportunistic infections
- Childhood asthma (resolved, no current treatment)

CURRENT MEDICATIONS

- None (ART not yet commenced – awaiting ID clinic)
- Paracetamol 1g PRN (self-medicating for fever)

ALLERGIES

Trimethoprim-Sulfamethoxazole (TMP-SMX) – rash (documented at HIV diagnosis review)

VITAL SIGNS

BP: 108/64 mmHg (borderline hypotensive) HR: 118 bpm (sinus tachycardia)
RR: 26 breaths/min (tachypnoeic) SpO2: 88% on room air -> 94% on 4L NC
Temperature: 39.6 degrees C Weight: 66 kg Height: 179 cm BMI: 20.6 kg/m2
CURB-65 Score: 2 (tachypnoea + urea elevated) – moderate severity CAP

LABS

Hb: 104 g/L (normocytic anaemia – HIV-related)
WBC: $3.2 \times 10^9/L$ (leucopaenia) Neutrophils: $2.1 \times 10^9/L$
Lymphocytes: $0.6 \times 10^9/L$ (low) Platelets: $98 \times 10^9/L$ (mild thrombocytopenia)
CRP: 218 mg/L (markedly elevated) Procalcitonin: 4.8 ug/L (elevated)
Urea: 11.2 mmol/L Creatinine: 104 umol/L eGFR: 88 mL/min/1.73m²
Na: 129 mmol/L (hyponatraemia) K: 3.3 mmol/L (hypokalaemia)
LDH: 612 U/L (markedly elevated – raises PCP suspicion)
Albumin: 22 g/L (hypoalbuminaemia – severe)
ABG (on 4L O₂): pH 7.43, pO₂ 8.1 kPa, pCO₂ 3.8 kPa, HCO₃ 21 – type 1 RF
CD4 count: 88 cells/uL (worsened from 112 at diagnosis)
Urine Pneumococcal antigen: Positive Urine Legionella antigen: Negative
Blood cultures x2: Pending Beta-D-glucan: Pending (PCP screen)

IMAGING

CXR: Right lower lobe consolidation with air bronchograms. Bilateral perihilar haziness. No pleural effusion. No pneumothorax. Left lung clear.
Pattern: right lower lobe pneumonia – cannot exclude concurrent PCP.

CT chest (with contrast):

Right lower lobe dense consolidation – lobar pneumonia pattern.
Bilateral ground-glass opacification predominantly perihilar and upper lobe – pattern consistent with concurrent *Pneumocystis jirovecii* pneumonia (PCP).
Mediastinal lymphadenopathy (largest node 1.8 x 1.4 cm) – reactive vs. lymphoma.
No pleural effusion. No cavitation. No empyema.

ASSESSMENT

Severe community-acquired pneumonia – *Pneumococcus* confirmed (urinary antigen +ve).
Concurrent PCP – clinically suspected (CD4 88, LDH 612, bilateral GGO on CT).
HIV infection – advanced (CD4 88 cells/uL), not yet on ART.
Sepsis – NEWS2 score 9, borderline haemodynamic compromise.
Hyponatraemia (SIADH pattern) – Na 129 mmol/L.
Hypokalaemia – K 3.3 mmol/L.

PLAN

1. Admit to Infectious Diseases ward – isolation room, droplet precautions.
2. IV Co-Amoxiclav 1.2g 8-hourly + Azithromycin 500mg once daily (CAP).
3. IV Clindamycin 600mg 8-hourly + Primaquine 30mg once daily for PCP (TMP-SMX allergy – alternative regimen). Continue until BAL confirms/excludes.
4. High-flow oxygen – maintain SpO₂ > 92%. ICU escalation if SpO₂ < 90%.
5. IV fluid resuscitation – Hartmann's 500mL bolus now, then reassess.
6. IV Potassium replacement – 40mmol KCl in 500mL over 4 hours.
7. Fluid restriction 1.5L/day – manage SIADH (Na 129 mmol/L).
8. ID consult – Dr. Priya Subramaniam to review ART initiation timing.
9. Bronchoscopy + BAL – 28 June 2026. Zainab Adeyemi (sister) to be consented.
10. Social work referral – housing, support, HIV disclosure counselling.
11. Notify public health – HIV case notification (statutory requirement).
12. Patient ID wristband: 9920384.

Signed: Dr. Aisha Kamara, ID Registrar

Date: 27 June 2026

Facility Contact: Northwestern Memorial Hospital, 251 E Huron St, Chicago IL 60611